

RadioTHERAPY

Insights Corner HIGHLIGHTS

Month: July 2026

Presenter: Team Octavus





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- ▶ Highlights from AACR-25 (**May, 25**)
- ▶ US Tariff impact on Pharma industries (**Jun, 25**)
- ▶ EULAR-25: Key insights of Psoriatic Arthritis (**Jul, 25**)
- ▶ Antibody Drug Conjugate: The next big investment in Oncology (**Aug, 25**)
- ▶ Animal Health (**Oct, 25**)
- ▶ Obesity deep dive (**Nov, 25**)
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- ▶ Hematological Highlights from ASH 2025 (**Jan, 26**)
- ▶ China Biotech's Global Positioning from JPM-26 (**Feb, 26**)
- ▶ BIOSIMILARS RISING: The Catalyst For A New Age In Biologics Therapies (**Mar, 26**)
- ▶ Cell Therapy Within CGT (**Apr, 26**)
- ▶ Gene Therapy Within CGT (**May, 26**)
- ▶ Regulatory Designations US FDA & EMA (**June, 26**)

Additional: **1 Beautiful Bill**

PRIOR ISSUES

Executive Summary

Radiopharmaceuticals are shifting from a niche modality to a strategic oncology growth engine with success hinged on capacity, supply, and integration

1

A Mainstream Oncology Platform

No longer a niche modality, Radiopharma is a core oncology growth engine, projected to grow from ~\$7.9B (2026) to ~\$17B (2035) at ~8.9% CAGR

2

Theranostics Drives Adoption

The “see it, treat it” model pairs imaging with matched therapy on one biomarker (led by Novartis LUTATHERA), and is expanding beyond PSMA/NETs into Fibroblast Activation Protein, Carbonic anhydrase IX, and GPCR targets

3

Supply Becomes the Competitive Advantage

Isotope production is concentrated in a handful of reactors, and alpha-emitter capacity lags projected demand, making supply security a true competitive barrier

4

Policy and Capital Align

Active US FDA/EMA guidance alongside more than \$13B in deals since 2023 signal that regulators and investors are both moving to accelerate commercialization

5

Integration Defines Winners

With just-in-time supply chains, limited treatment-center capacity, and complex reimbursement, advantage will favour players who vertically integrate manufacturing, logistics, and care networks (e.g. ITM's Lu-177 supply + ITM-11 pipeline)



Foundations of Radiopharmaceuticals

Objective: Establish a fundamental understanding of radiopharmaceuticals, theranostics, and their role in precision medicine



Radiopharmaceuticals: Foundational Concepts

Overview, mechanism of action and clinical applications of radiopharmaceuticals

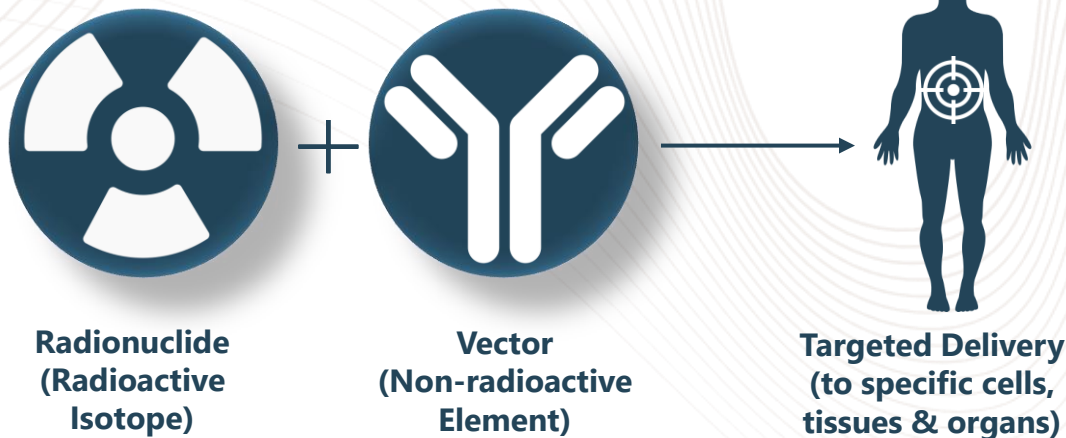


Overview

According to WHO and IAEA, Radiopharmaceuticals are specialized medicinal formulations that contain radioisotopes and are used for the diagnosis and treatment of diseases across a wide range of clinical applications

The Science behind Radiopharmaceuticals

Radiopharmaceuticals enable accurate delivery of radionuclides (radioactive isotope) linked to a vector (non-radioactive element) such as small molecules, peptides, and antibodies that target specific cells, tissues or organs in the body



Key Radioisotopes in clinical use



Technetium-99m

used in a multiple diagnostic scans such as bone scans, kidney function tests and MPI



Fluorine-18

used in PET imaging to diagnose cancer



Iodine-131

used to treat thyroid cancer



Lutetium-177

used to treat neuroendocrine tumours



Abb: IAEA: International Atomic Energy Agency ; MPI: Myocardial Perfusion Imaging; PET: Positron Emission Tomography; WHO: World Health Organization

Theranostics: Bridging Diagnosis and Therapy

Understanding the integration of molecular imaging and targeted radionuclide therapy to support precision medicine



One Target

Diagnostic and Therapeutic agents bind to the same biomarker for a unified approach



Personalized care

Imaging defines target expression and disease biology, guiding personalized therapy decisions



Real-Time Assessment

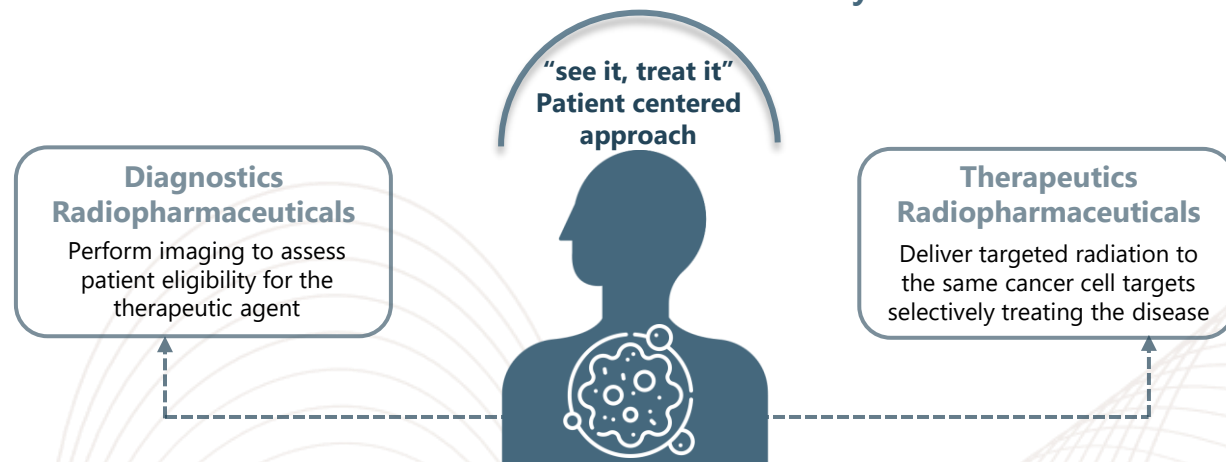
Enable assessment of disease activity and therapeutic response to guide ongoing patient care



Better Outcomes

Facilitate more targeted and efficient treatment approach, reducing adverse effects and improving clinical outcomes

The Radiotheranostic Pathway



From Separate Pathways to Integrated Care

Traditional Approach	Radiotheranostic Approach
Conventional cancer treatment often involves distinct diagnostic and treatment processes, potentially resulting in treatment delays and a more complex patient journey	Radiotheranostic integrates a diagnostic and a therapeutic radiopharmaceutical that act on same biomarker for simultaneous detection and treatment of diseases
 More delays, Greater complexity	 One Target, Two solutions



A Growing Global Impact



Oncology reviews describe an "exponential, global expansion" of radiotheranostics driven by its ability to target and eliminate tumor cells with relatively limited off target toxicity



ESMO and other expert now present radioligand therapy (RLT) as a promising and increasingly relevant modality for the treatment of advanced & refractory cancers

Regulatory & Company Landscape

Objective: Highlight the evolving regulatory environment and showcase Novartis leadership in advancing radioligand therapy



Regulatory Landscape of Radiopharmaceuticals (2024-2026)

Recent guidance, policy updates, and regulatory initiatives from the US FDA and EMA

FDA
U.S. FOOD & DRUG
ADMINISTRATION

**Dec 2025
(Draft)**

Guidelines for the quality of radiopharmaceuticals

This guideline outlines the additional chemical, pharmaceutical, and biological information required for radiopharmaceuticals based on synthetic chemical substances when submitting marketing authorization applications or variations to authorized medicinal products.

Dosage optimization for therapeutic radiopharmaceuticals in oncology during clinical development

**Aug 2025
(Draft)**

This guidance helps sponsors determine the optimal dosing regimen including administered activity and treatment schedule for oncology radiopharmaceutical therapies (RPTs) during clinical development before submitting a marketing application for a new indication.
Note: It does not cover the selection of the initial administered activity for first-in-human RPTs trials or other aspects of RPTs clinical development, such as dosimetry software, fixed versus personalized dosimetry-based dosing, and theranostic co-development.



EUROPEAN MEDICINES AGENCY
SCIENCE · MEDICINES · HEALTH

Oct 2024

Concept paper on the clinical evaluation of therapeutic radiopharmaceuticals in oncology

This concept paper aims to identify aspects unique to therapeutic radiopharmaceuticals (tRPs) that should be addressed in a future guideline, complementing the existing "EMA Guideline on the Clinical Evaluation of Anticancer Medicinal Products". The proposed guideline will provide specific recommendations on applying relevant regulatory principles to the clinical development of tRPs for marketing authorization applications.

NOVARTIS: Global Leader in Radioligand Therapy



Exploring Novartis overview, clinical pipeline, commercial portfolio, and strategic vision

Overview

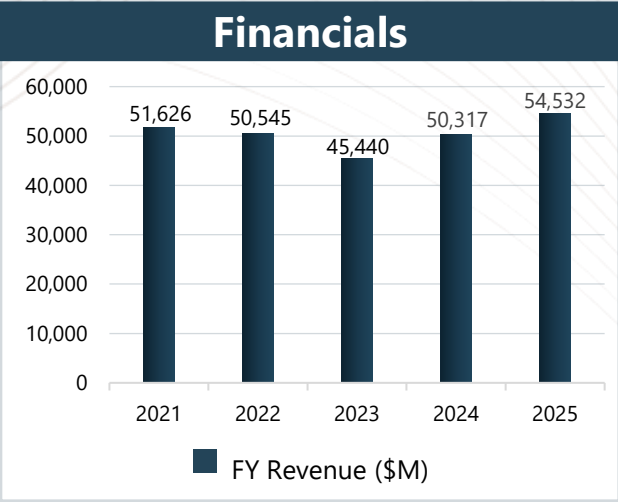
 Headquarter: Basel, Switzerland

 Business: Innovative medicines

 Therapeutic Focus: CVRM;
Immunology; Neuroscience; Oncology

 Strategic Oncology Platform:
Radioligand Therapy (RLT)

 Revenue (2025): \$54.5B



Clinical Pipeline (Phase 3 & Registration)

Asset	Indication	Target	ROA	Planned filling dates	Phase
AAA617	mHSPC			-	Registration
AAA617	Oligometastatic prostate cancer			≥2029	Phase 3
AAA817	mCRPC; 1L			≥2029	Phase 3
AAA817	post Lu mCRPC			2028	Phase 3
AAA601	GEP NET			2028	Phase 3

Marketed RLTs

Approved Products	Target	Radioisotopes	Approved Indication (Across US & EU)	Revenue (Q1'26) \$M
 LUTATHERA (lutetium Lu 177 dotatate) injection, for intravenous use	SSTR	lutetium 177	SSTR-positive GEP NET	211
 PLUVICTO lutetium Lu 177 vipivotide tetraxetan injection for intravenous use	PSMA	lutetium 177	PSMA-positive mCRPC	642

Patent Protection

Products		
LUTATHERA	2029 (ODE expired)	2029
PLUVICTO	2034	2037

Octavus Insight

- Novartis is consolidating leadership in RLT, with focused late-stage assets around SSTR and PSMA that deepen its control of GEP NET and prostate cancer segments
- Strong and growing RLT revenues (PLUVICTO, LUTATHERA) indicate RLT is now a core growth driver, enabling further investment that could entrench Novartis' therapies as standards of care in these indications

 SSTR

 PSMA

 IV Infusion

 US

 EU

Abb: CVRM: Cardiovascular, Renal and Metabolism; GEP NET: Gastroenteropancreatic Neuroendocrine Tumors; mCRPC: Metastatic Castration-Resistant Prostate Cancer; mHSPC: Metastatic Hormone-Sensitive Prostate Cancer; PSMA: Prostate Membrane Specific Antigen; SSTR: Somatostatin Receptor

Market Opportunity & Commercial Landscape

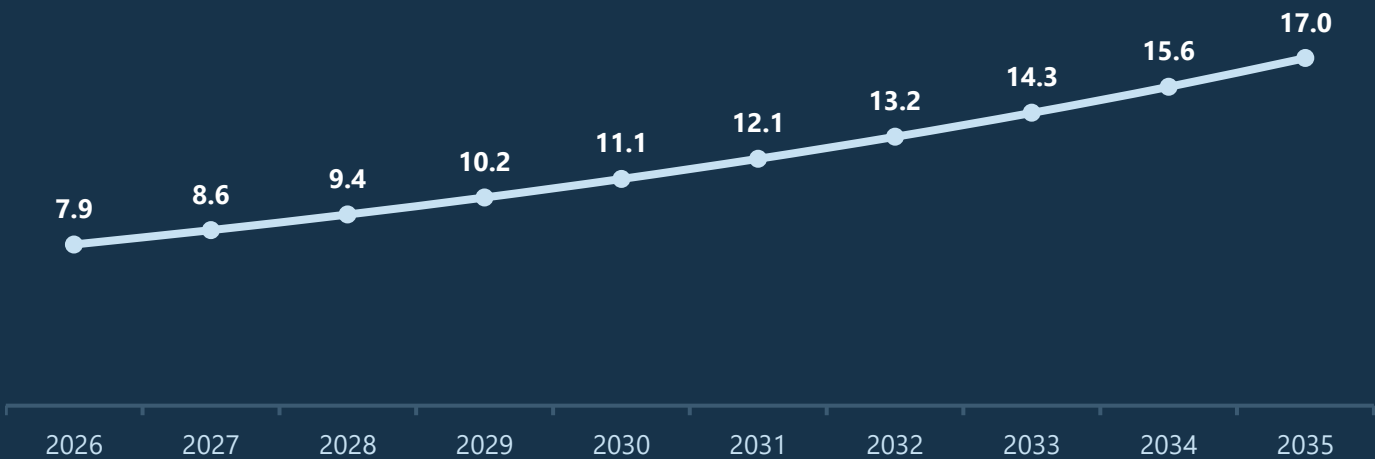
Objective: Assess the market potential, commercialization pathway, reimbursement environment, and key barriers influencing radiopharmaceutical adoption



Radiopharmaceutical Market Snapshot (2026-2035)

Assessing market size, growth drivers, competitive advantage, and barrier in the global radiopharmaceutical sector

MARKET SIZE (\$B)



\$17B

Projected market size by 2035

8.9%

CAGR, 2026–2035

Asia-Pacific

Fastest growing region

KEY HIGHLIGHTS



Growth Drivers

Oncology is the largest application, with North America leading and Asia-Pacific emerging as the fastest growing region on expanding nuclear medicine infrastructure



Industry Landscape

Technetium-99m continues to dominate usage, while Gallium-68 is the fastest-growing isotope; recent M&A activity reflects strong investment in isotope production and theranostic capabilities

KEY INSIGHTS



Market Transition

Views differ on the diagnostic vs therapeutic split, but theranostics is accelerating the shift toward targeted radiopharmaceutical therapies



Competitive Advantage

GE HealthCare's and AstraZeneca's recent acquisitions show isotope supply and manufacturing are now as strategic as drug discovery itself



Key Barrier

Isotope supply constraints and short radionuclide half-lives raise barriers to entry, favoring firms with established production networks

Radiopharmaceuticals: What Could Hinder Market Growth

1

Isotope Supply Concentration

Only 8-10 of ~60 active global research reactors are the major producers of medical radionuclides

A reactor going offline is a recurring risk, not a rare one, any pipeline asset depends on its isotope sourcing plan

2

Infrastructure & Workforce Capacity Constraints

An estimated 70-280 additional RPT treatment centers are needed for ~41,500 eligible NET/Prostate Cancer patients. Workforce shortages and unequal distribution limit access to RPT

Market expansion is constrained by treatment-center capacity and specialist availability, not just regulatory approval or reimbursement

3

Regulatory Pathway Complexity

Dual regulatory pathways for drug and radiation safety complicate radiopharmaceutical review; US FDA review involves multiple divisions with NRC oversight

Requires coordinated regulatory, manufacturing, and radiation-safety planning

4

Alpha-Emitter Supply Risk

Limited supply of alpha-emitting isotopes (eg: Pb-212 and Ac-225) is driving companies such as Perspective Therapeutics and Actinium Pharmaceuticals to invest in proprietary production capabilities

Decentralized isotope production is emerging as a competitive differentiator, particularly among smaller and mid-cap developers

5

Adverse Event Underreporting

Adverse event underreporting is driven by time constraints, low staff awareness and weak reporting culture; 43% of reported ADRs in a French database were serious, including 12 deaths

Low reported ADR rates should be interpreted with caution due to potential underreporting

6

Emerging Generic Competition

PNT2003 (radioequivalent to LUTATHERA) received tentative US FDA approval, with final approval pending patent litigation

First-generation RPTs may begin to face pricing and market-share pressures as generic competition emerges

Abb: ADR: Adverse Drug Reaction; NET: Neuroendocrine Tumor; NRC: Nuclear Regulatory Commission; RPT: Radiopharmaceutical Therapy

This presentation is non-exhaustive. For additional details or clarifications, please feel free to contact us at bd@octavusconsulting.com

Radiopharmaceuticals Market Access

Payer economics, cost-effectiveness arguments and structural hurdles unique to radioactive drugs



Payer Dynamics

- ❖ **Diagnostic radiopharmaceuticals:** Under Medicare OPPTS, CMS separately reimburses qualifying diagnostic RPTs above the annual packaging threshold (\$630 in CY2025; \$655 in CY2026), while products below the threshold continue to receive packaged payment
 - Diagnostic RPTs above the \$655/day threshold are reimbursed using Mean Unit Cost (MUC), as CMS declined to adopt ASP-based payment for diagnostics in the CY2026 final rule
- ❖ **Therapeutic radiopharmaceuticals:** CMS reimburses payable therapeutic RPTs at ASP+6% with novel RPTs requiring appropriate HCPCS/ APC codes and clear payment methodology

High-cost diagnostic radiopharmaceuticals are paid separately using mean unit cost, while therapeutic agents are reimbursed at ASP+6% when appropriately coded



Cost-Effectiveness

- ❖ Budget impact models are mandatory in Spain and Italy, while cost-effectiveness models are strongly recommended to support pricing decisions. Although not mandatory, RWE can help demonstrate long-term outcomes and support sustainable pricing
- ❖ Eg: LUTATHERA was recommended by NICE under a confidential commercial arrangement, whereas PLUVICTO received a negative recommendation in its initial NICE appraisal, illustrating the importance of strong clinical and economic evidence in reimbursement decisions
- ❖ Cost-effectiveness models in the US focus on how RPTs reduce non-beneficial high-cost treatments and downstream toxicity, framing value in terms of budget impact and financial toxicity reduction rather than formal HTA

Country-specific economic evidence, especially budget impact and cost-effectiveness determines reimbursement success more than clinical data alone



Structural Hurdles Unique to Radioactive Drugs

- ❖ Radiopharmaceuticals have no inventory buffer, unlike conventional drugs, their raw material and final product have only a few days of shelf life, forcing a 'just-in-time' supply chain, leaving providers unable to buffer against inadequate reimbursement the way other drug classes can
- ❖ An estimated 70-280 additional RPT treatment centers are needed for ~41,500 eligible patients, while specialized infrastructure (authorized user physicians, radiation safety oversight & dedicated facilities) continues to constrain delivery capacity

Approval & reimbursement alone do not guarantee patient access, commercial scale depends on treatment-center capacity, specialized infrastructure, licensing and a trained workforce

Innovation & Clinical Development

Objective: Evaluate emerging therapies, recent clinical evidence, and approved products to understand the evolving radiopharmaceutical pipeline



Clinical Development: Radiopharma at ASCO 2026

Phase 1/2a targeted alpha therapy data in heavily pre-treated, MC1R-positive metastatic melanoma



Company Snapshot

Clinical-stage radiopharmaceutical company advancing targeted alpha therapies using its proprietary Pb-212 platform, integrating theranostic imaging and treatment to improve patient selection and precision tumor targeting

Results at a Glance: [212Pb]VMT01 Phase 1/2a (Melanoma)

Efficacy Endpoints	
Endpoint	Result
Evaluable Patients	22
Complete Response (CR)	0
Partial Response (PR)	1 (4.5%)
Stable Disease (SD)	9 (40.9%)
Progressive Disease (PD)	12 (54.5%)
Disease Control Rate (CR + PR + SD)	45.5% (10/22)
Progression-Free Survival	4 patients achieved PFS ≥6 months
Antitumor Activity	Observed primarily at the 3 mCi dose level

Safety Endpoints	
Endpoint	Result
Dose-Limiting Toxicities (DLTs)	None reported
Serious Adverse Events (SAEs)	All SAEs (including one Grade 5 SAE) were attributed to disease progression rather than treatment
Grade 1 TEAEs	19.2%
Grade 2 TEAEs	34.6%
Grade 3 TEAEs	8/26 patients (30.8%): 5 on monotherapy and 3 on 3 mCi + nivolumab
Overall Tolerability	Generally safe and well tolerated

 Early clinical responses in heavily pre-treated melanoma, combined with a favorable safety profile, provide initial clinical validation of Perspective's Pb-212 targeted alpha therapy platform beyond its lead NET program

Clinical Development: Radiopharma at AACR 2026

Updated Phase 1/2a interim data across three dose cohorts in SSTR2-positive neuroendocrine tumors



Company Snapshot

Clinical-stage radiopharmaceutical company advancing targeted alpha therapies using its proprietary Pb-212 platform, integrating theranostic imaging and treatment to improve patient selection and precision tumor targeting

Results at a Glance: [212Pb]VMT-α-NET Phase 1/2a (GEP-NETs), data cut-off Mar 4, 2026, Efficacy Population-25 patients (Cohort 1: n=2; Cohort 2: n=23)

Efficacy Endpoint	Result	Comments	Safety Endpoint	Result
Objective Response Rate (ORR)	43% (10/23)	Investigator-assessed RECIST v1.1 in first half of Cohort 2	Patients Evaluated	64
Complete Responses (CR)	0	No complete responses reported	Dose-Limiting Toxicities (DLTs)	None observed
Partial Responses (PR)	10 patients	Includes one additional new confirmed responder since ASCO GI 2026	Grade 5 Treatment-Related Adverse Events	None
Disease Control	High	Most remaining patients achieved stable disease	Serious Renal Toxicity	None reported
Tumor Shrinkage	Continued deepening	9 patients experienced further tumor shrinkage since the ESMO 2025 analysis	Grade ≥3 Treatment-Emergent Adverse Events (TEAEs)	23 patients (36%)
Progression-Free and Alive	18/25 patients (72%)	Across efficacy-evaluable Cohorts 1 and 2 at latest follow-up	Overall Tolerability	Generally, well tolerated across dose cohorts
Durability	Responses continued to deepen over time	Follow-up remains ongoing		



Deepening responses, a 72% progression-free-and-alive rate, and a favorable safety profile across dose escalation provide early clinical validation of Perspective's Pb-212 targeted alpha platform as its lead NET program advances toward a registration-enabling study

Abb: GEP NET: Gastroenteropancreatic Neuroendocrine Tumors

Product Profile: ITM-11 | Investigational Product

An investigational radiopharma designed for GEP-NETs (Oncology)

Product: ITM-11 (Solucin/ n.c.a. ¹⁷⁷Lu-edotreotide/ n.c.a. ¹⁷⁷Lu-DOTATOC)

Indication: Grade 1/2, SSTR+ GEP-NETs

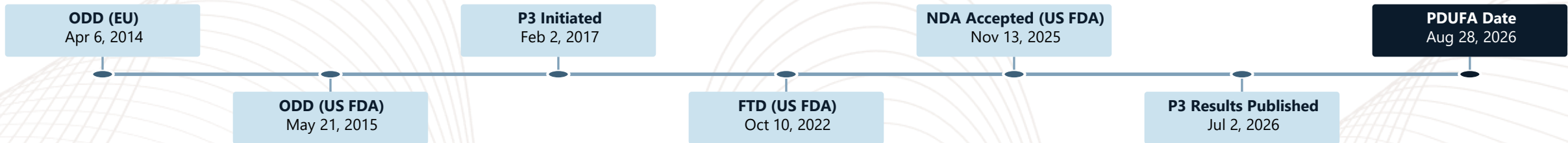
Mechanism of Action: PRRT-SSTR2 agonist labeled with n.c.a. ¹⁷⁷Lu

Route of Administration: Intravenous infusion (IV)

Dose / Regimen: 7.5 ± 0.7 GBq IV q3mo (≤4 cycles), with nephroprotective amino acids



Regulatory Milestone Timeline



Note: ITM-11 is investigational - not yet approved by any regulatory authority

Key Insights

- ✓ COMPETE trial beat Everolimus, but ITM-11’s (a beta radioisotope) real competitor once approved is LUTATHERA - same isotope, same target, same indication, same infrastructure
- ✓ Unlike PLUVICTO (drug bought first, isotope supply chased after), ITM built the therapy on its own existing Lu-177 manufacturing base, a vertically integrated model
- ✓ If approved in Aug 2026, ITM-11 would be the first new RPT since PLUVICTO (2022); the open question is whether it expands the capacity-constrained PRRT infusion grid or redistributes existing Lu-177 volume

Risk to watch

- ✓ ITM-11 showed a small survival edge (63.4 vs 58.7 months), but it wasn't statistically significant, patient crossover may be why. A similar issue delayed PLUVICTO (a different radiopharmaceutical for Prostate Cancer) by more than a year

Key Results

Regimen	ITM-11 (n.c.a. ¹⁷⁷ Lu-edotreotide)		Everolimus (SoC)
Trial ID	COMPETE (NCT03049189)		
Phase	P3		
Line of Therapy	1L/2L		
Dose	7.5 GBq IV, q3mo, ≤4 cycles	10mg PO daily, ≤30mo	
(Patients Dosed) n	207	102	
mPFS (mos)	23.9	14.1	
ORR (%)	22	4	
Gr 3/4 TRAEs (%)	18	40	
Source	The Lancet (Jul 2026); ITM press releases		

Abb: GEP-NET: Gastroenteropancreatic Neuroendocrine Tumor; FTD: Fast Track Designation; NDA: New Drug Application; ODD: Orphan Drug Designation; PDUFA: Prescription Drug User Fee Act ; PRRT: Peptide Receptor Radionuclide Therapy; SSTR: Somatostatin Receptor;

Product Profile: AMYVID | Approved Product

An approved radiopharma designed for Alzheimer's disease (beyond oncology role of radiopharmaceutical diagnostics)



Generic Name	Florbetapir F 18
Diagnostic Area	Alzheimer's disease / cognitive decline
MoA	Binds fibrillar Aβ aggregates (Kd≈3.7nM)
Molecular Target	β-amyloid neuritic plaques
Biomarker	Cerebral amyloid burden (SUVR/Centiloid)
Radioisotope	Fluorine-18
Modality	PET (brain)
Route of Admin.	IV bolus injection
Dose	370 MBq (10 mCi), ≤10 mL
Imaging Time	~30–50 min post-injection
Originator	Avid Radiopharmaceuticals
Manufacturer	Eli Lilly & Co.
Commercial Partners	PD Radiopharma (Japan)

Key Clinical Evidence

92–100%

Sensitivity / Specificity vs. autopsy (≤2 yrs)

- Pivotal autopsy study (Clark et al., Lancet Neurol 2012, n=59) vs. CERAD histopathology, basis for FDA/EMA approval
- JAMA 2011 (Clark et al.), first prospective imaging-to-autopsy correlation for an amyloid PET tracer

96–100%

Sensitivity / Specificity vs. autopsy (≤1 yr)

Key Limitations

- Not diagnostic of Alzheimer Disease alone, requires full clinical work-up
- Plaques also occur in other neurodegenerative disease & cognitively normal elderly
- Requires certified reader training; not validated to monitor treatment response

Competitive Positioning

Attribute	Amyvid	Neuraceq	Vizamyl
FDA Approval	2012 (1st)	2014	2013
Sens./Spec. (autopsy)	92%/100%	98%/80%	92%/88%
Imaging Window	30–50 min	45–130 min	60–90 min
Read Format	Binary visual	Binary visual	Binary + color

Key Takeaways

- First US FDA-approved amyloid PET tracer (2012)
- Autopsy-validated diagnostic for AD-related decline
- Widely used to confirm / rule out amyloid pathology
- Emerging patient-selection tool for anti-amyloid therapy
- Broad regulatory footprint: US, EU, Japan

Competitive Intelligence & Strategic Insights

Objective: Analyze partnerships, funding trends, and the competitive landscape to identify strategic opportunities and emerging industry trends



Radiotherapies Landscape (US only)

An overview of the US radiotherapy landscape, highlighting approved therapies, and clinical pipeline

Radioisotope	Phase 2		Phase 3 / Reg				Approved	
177-Lu	TLX597-Tx  PSMA+ mHSPC	 (lutetium Lu 177 dotatate) injection, for intravenous use  GEP-NETs (Pediatrics)	ITM-11  GEP-NETs (G2/G3)	TLX591-Tx  1L & 2L mCRPC	 (lutetium Lu 177 dotatate) injection, for intravenous use  GEP-NETs	 (lutetium Lu 177 vipivotide tetraxetan) injection, for intravenous use  mHSPC	 (lutetium Lu 177 vipivotide tetraxetan) injection, for intravenous use  PSMA+ve mCRPC	 (lutetium Lu 177 dotatate) injection, for intravenous use  SSTR+ve GEP-NETs
	TLX250-Tx  ccRCC	AAA614  Solid Tumor	ITM-11  Lung NET	 (lutetium Lu 177 vipivotide tetraxetan) injection, for intravenous use  omPC	ITM-11  GEP-NETs (G1/G2)	PNT2003*  GEP-NETs		
225-Ac	FPI-2265  A member of the AstraZeneca Group PSMA+ve Prostate Cancer	RYZ801  A Bristol Myers Squibb Company HCC	Actimab-A  r/r AML	RYZ101  A Bristol Myers Squibb Company GEP-NETs	AAA817  Post Lu- mCRPC	AAA817  mCRPC		
131-I	Iopofosine  DLBCL, MM, NHL	TLX101-Tx  GBM			Iopofosine  Waldenström macroglobulinemia	Iomab-B  Improve BMT	 danaparacal  Pheochromocytoma/ Paraganglioma	
90-Yt		TLX66-Tx  HSCT & Leukaemia					 (britumomab tiuxetan) injection, for intravenous use  BIOPHARMA 1L & r/r NHL	
Other	Radspherin 224-Ra  Ovarian Cancer	CLR 121125 125-I  Solid Tumor- TNBC			SAR-bisPSMA 64/67-Cu  PHARMACEUTICALS mCRPC	AlphaMedix 212-Pb  RadioMedix SSTR+ GEP-NETs	 ONCOLOGICALS  Bone metastases mCRPC	CLR 121125 153-Sm  QUADRAMET (Samarium 153 LEONORAN INJECTION) Bone Pain due to Bone Metastasis
	VMT-α-NET 203-Pb  THERAPEUTICS NETs	Cu64 PSMA 64-Cu  LIFE FORWARD Metastatic Prostate Cancer						

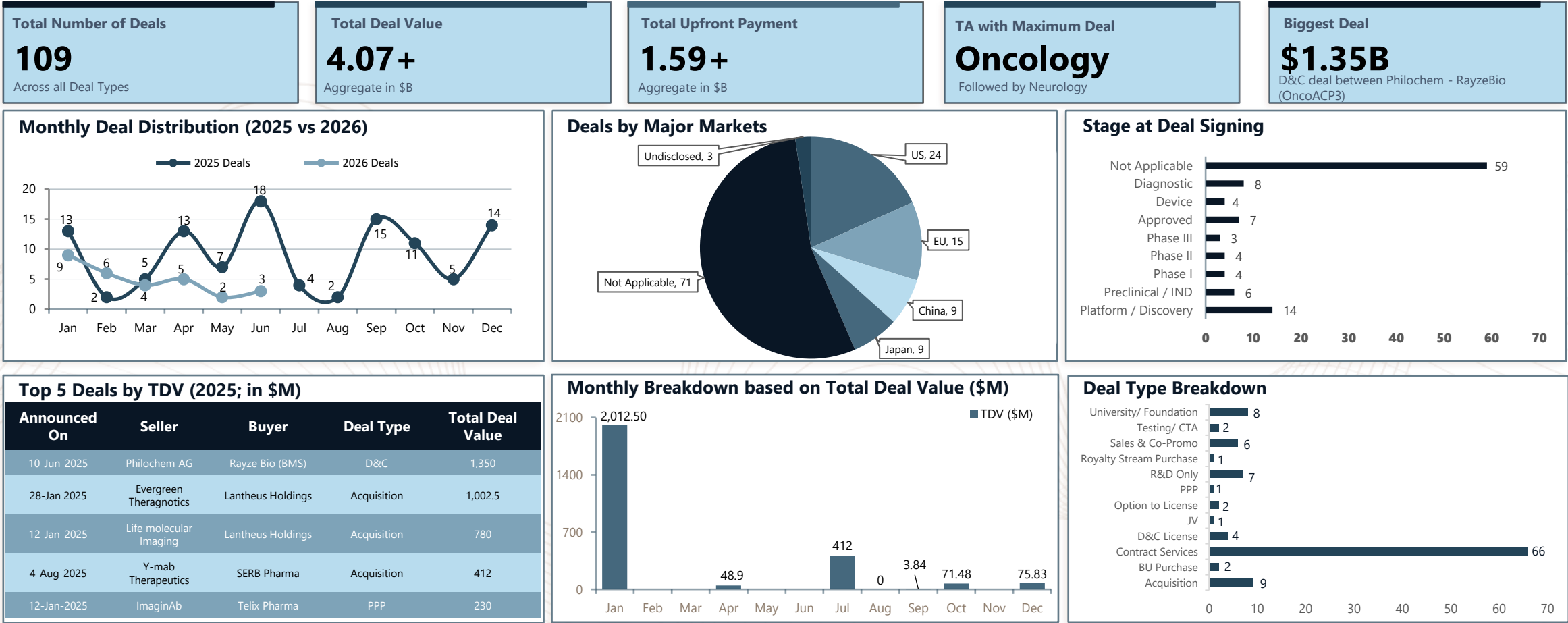
Abb: CRPC: Castration Resistant Prostate Cancer; GEP-NET: Gastroenteropancreatic Neuroendocrine Tumor; HSPC: Hormone-Sensitive Prostate Cancer; RCC: Renal Cell Carcinoma

R: Registration; Broken border: Planned Trial

* Received tentative approval in Mar 2, 2026 awaiting legal resolution

Radiopharma Deals Dashboard (2025)

Analyzing the 2025 radiopharma deal landscape through TDV, market distribution, development stage, and deal type



Key Learnings & Insights

Lantheus Holding is positioning itself as the global radio-therapy and radio-diagnostic leader

US remains the primary market followed by EU within the disclosed geographies

Platform/ Discovery stage remains the area of highest and most deals which shows promise and interest in the field

As many don't have specialized facilities and requires lot of funding. And as the field grows contract service deals are higher and gathering traction

Among the Top 10 Pharma companies Novartis lead the way with multiple molecules across isotopes and phases and indications

Abb: CTA: Clinical Testing Agreement; D&C: Development and Commercialization; IND: Investigational New Drug; JV: Joint Venture; PPP: Platform, Product & Purchase; R&D: Research & Development; TDV: Total Deal Value; US: United States

Radiopharma Funding Dashboard (2025)

Tracking capital investments, funding rounds, therapy focus and investor activity in the radiopharmaceutical sector

Total Number of Rounds 21 Across all Founding Rounds	Total Funds Generated 1.48+ Aggregate in \$B	Common TA Oncology 18 Companies focused on Oncology	Max Type Round PIPE Type of rounds maximum number of companies	Biggest Funding Round \$365.4B Aktis Oncology (IPO)
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Date Announced	Company Name	Amount (\$M)	Round	Primary TA (Lead Phase)	Stage of Funding	Lead Investor
10-Jan-2025	Radiopharm Theranostics	5	PIPE	Oncology	Diagnostic	Lantheus Holdings
31-Jan-2025	AdvanCell Isotopes	112	Series C	Oncology	Phase 1	SV Health Investors
11-Feb-2025	United Theranotics	15	Debt/ Loan	Oncology	N/A	Old National Bancorp.
4-Mar-2025	Plus Therapeutics	15	PIPE	Autoimmune	Phase 2	NA
28-Apr-2025	ITM Isotope Technologies	140	PIPE	Oncology	Device	Oramed Pharmaceuticals
27-May-2025	Alpha Tau Medical	36.8	Debt/ Loan	Oncology	Approved	Blue Owl Capital
2-Jun-2025	ASP Isotopes	50	PIPE	Not Applicable	Not Applicable	Undisclosed
6-Jun-2025	Accuray	82	Convertible Notes	Oncology	Diagnostic	Undisclosed
2-Jul-2025	Primo Biotechnology	7.58	Series A	Oncology	Preclinical/ IND	Undisclosed
9-Jul-2025	Actithera	75.5	Series A	Oncology	Preclinical/ IND	M Ventures (Merck KGaA)
10-Jul-2025	Nuclidium AG	132.6	Series B	Oncology	Phase 1	Kurma Growth Opportunities Fund
27-Jul-2025	Zonsen PepLib Biotech	13.93	Series B	Oncology	Preclinical/ IND	CDH Investments
29-Jul-2025	Alpha Nuclide	NA	Series A	Oncology	Not Applicable	NA
29-Jul-2025	ArtBio	132	Series B	Oncology	Phase 2	Sofinnova Investments
9-Sep-2025	Ridge Biotechnologies	25	Seed	Oncology	Platform/ Discovery	Sutter Hill Ventures
23-Sep-2025	Monopar Therapeutics	135	PIPE	Oncology	Phase 3	Undisclosed
24-Sep-2025	Leo Cancer Care	40	Series Unspecified / Private	Oncology	Diagnostic	Catalio Capital Management
29-Sep-2025	Full-life Technologies	77 (50+27)	Series C; Debt/ Loan	Oncology	Phase 1	Junson Capital
20-Oct-2025	Radiopharm Theranostics	22.7	PIPE	Oncology	Diagnostic	Undisclosed
19-Dec-2025	Aktis Oncology	365.4	IPO	Oncology	Phase 1	Undisclosed

Radiopharma on the Rise: Trends Defining the Next Era

Objective: Provide an overview of the major forces driving radiopharmaceutical innovation, including isotope evolution, regulatory momentum, dealmaking activity, and future growth opportunities



Emerging Trends & Future Outlook

Isotope trajectory, approval footprint, dealmaking and where the field is heading



Gamma Emitters Remain the Workhorse

- Tc-99m still drives ~80% of diagnostic scans
- CZT-SPECT lifts sensitivity 7x, resolution 2x

80% of diagnostic volume



Alpha Emitters Lead on Momentum

- Ac-225/Pb-212 cut off-target toxicity, beat resistance
- Global Ac-225 production was ~1,800 curie/year (2023)

Projected clinical demand of 5,000–8,000 curie/year by 2030



AI Dosimetry & Digital Twins

- Machine learning can predict absorbed dose before therapy
- EANM/ICRU standards fuel adoption

Next personalization frontier



Approval Footprint Stays Narrow

- Only 5 of 20 EU approvals are therapeutic
- Lutathera & Pluvicto lead oncology use

20 EU-approved radiopharmaceuticals



Beyond-Oncology Deals Signal Expansion

- Lilly-Aktis deal worth up to \$1.1B for tumor-targeting platform
- Radionetics buy-option adds GPCR-targeted portfolio for ~\$1B

\$1.1B Lilly–Aktis deal



M&A and Manufacturing Consolidation

- More than \$13B in Radiopharma deals since 2023 (BMS, AstraZeneca, Lilly, Novartis)
- More than 80% of pipeline cites Ac-225 supply as bottleneck

\$13B+ in deals since 2023



Next Decade: Earlier, Broader, Multi-Isotope

- More than 400 trials target FAP, CAIX, GPR beyond PSMA/NETs
- Market projected to grow from \$7.9B (2026) to \$17B by 2035

~8.9% CAGR through 2035

ABOUT COMPANY

Research & Intelligence that Proves our ROI



Competitive Intelligence & Monitoring

- Real-time CI monitoring
- Strategic early warning
- War-gaming & Simulation workshops
- Conference coverage
- Trial Optimization Strategy
- Digital Biomarker Strategy
- Competitor Promotion tracking



Portfolio & Commercial Strategy

- Opportunity Scanning
- Portfolio Strategy & Asset Prioritization
- Brand positioning
- Peak Sales Forecasting
- Digital therapeutic evaluation
- Innovation pipeline screening



Business Development & Licensing

- Target identification and validation
- Retrospective deal analysis
- Synergy assessment
- M&A due diligence
- Digital health and innovation advisory



Market Access & Evidence

- Payer landscape mapping
- Value dossier support
- RWE strategy design
- Pricing and reimbursement roadmaps
- RWE Analysis
- Medical Insights Research
- KOL identification and mapping
- KOL Engagement strategy



Medical Affairs & Analytics

- HCP & Patient insights
- Field force analysis
- Forecasting
- Integrated data analytics
- Patient journey mapping & analytics
- Patient Advocacy mapping
- Content Strategy
- Digital Sentiment analysis



Capabilities are configured to fit the decision, asset stage and timeline.

Continue Exploring with Octavus

How We Support Decision Makers

Competitive Intelligence

Market Landscape Assessments

Clinical Pipeline Intelligence

Licensing & Deal Intelligence

KOL Mapping

Conference Intelligence

Market Access Strategy

Commercial & Portfolio Strategy

Competitive Intelligence in Oncology

Supported a global pharma company in redesigning its competitive intelligence framework to strengthen portfolio and market monitoring decisions.

[Read Case Study](#)

Clinical Trial Intelligence

Built a global clinical trial intelligence dashboard covering anti-PDx molecules to support recruitment strategy, site selection, and competitive benchmarking.

[Read Case Study](#)

Conference Coverage

Explore executive summaries and scientific highlights from major global healthcare conferences, translated into decision-ready intelligence.

[View Sample Report](#)

Explore Our Capabilities

Learn how Octavus supports competitive intelligence, market research, due diligence, commercial strategy, analytics, and healthcare consulting.

[Explore Here](#)

Explore related case studies, conference intelligence, and strategic capabilities supporting life sciences decision-makers.

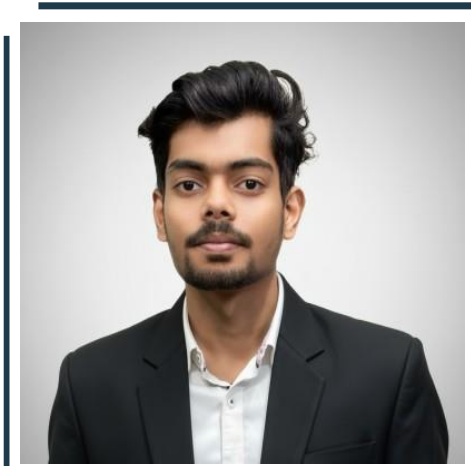
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